

HEALTH SYSTEMS, PUBLIC HEALTH AND UNIVERSAL HEALTH COVERAGE

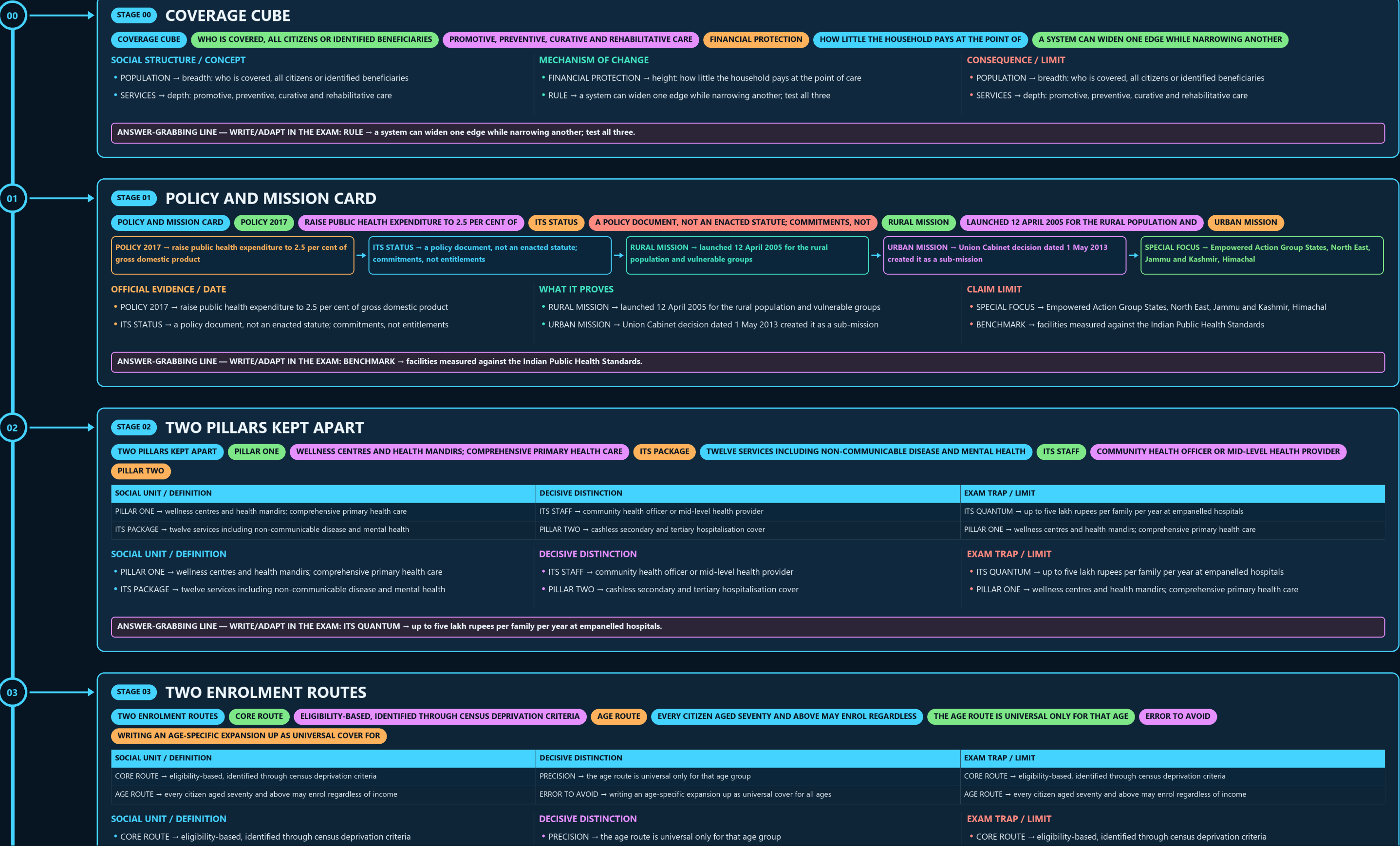
COVERAGE CUBE → POLICY AND MISSION CARD → TWO PILLARS KEPT APART → TWO ENROLMENT ROUTES → SOURCE DISCIPLINE CARD → IMPOVERISHMENT MECHANISM

Read the thick cyan rail from Stage 00 to the final core synthesis. Each card uses a concept-appropriate structure; the grey E card is optional enrichment only and never repairs missing core content.

PRIMARY CORE FLOW SUBORDINATE EXTRA APPROVAL / REVIEW

Approval: FALSE • Pending user review • active generation g1 • explicit approval of this exact generation is required • all prior artifacts unchanged

CORE BEFORE EXTRA • prior artefacts unchanged • exact same master drives poster and tiles



03

STAGE 03 TWO ENROLMENT ROUTES

TWO ENROLMENT ROUTES CORE ROUTE ELIGIBILITY-BASED, IDENTIFIED THROUGH CENSUS DEPRIVATION CRITERIA AGE ROUTE EVERY CITIZEN AGED SEVENTY AND ABOVE MAY ENROL REGARDLESS OF INCOME THE AGE ROUTE IS UNIVERSAL ONLY FOR THAT AGE ERROR TO AVOID

WRITING AN AGE-SPECIFIC EXPANSION UP AS UNIVERSAL COVER FOR

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
CORE ROUTE → eligibility-based, identified through census deprivation criteria	PRECISION → the age route is universal only for that age group	CORE ROUTE → eligibility-based, identified through census deprivation criteria
AGE ROUTE → every citizen aged seventy and above may enrol regardless of income	ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages	AGE ROUTE → every citizen aged seventy and above may enrol regardless of income
SOCIAL UNIT / DEFINITION - CORE ROUTE → eligibility-based, identified through census deprivation criteria - AGE ROUTE → every citizen aged seventy and above may enrol regardless of income	DECISIVE DISTINCTION - PRECISION → the age route is universal only for that age group - ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages	EXAM TRAP / LIMIT - CORE ROUTE → eligibility-based, identified through census deprivation criteria - AGE ROUTE → every citizen aged seventy and above may enrol regardless of income

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: ERROR TO AVOID → writing an age-specific expansion up as universal cover for all ages.

04

STAGE 04 SOURCE DISCIPLINE CARD

SOURCE DISCIPLINE CARD HEALTH AUTHORITY IMPLEMENTS THE HOSPITALISATION PILLAR; EMPANELS AND PAYS CLAIMS HEALTH ACCOUNTS OFFICIAL ESTIMATES OF PUBLIC, PRIVATE AND OUT-OF-POCKET SPENDING 47.1 PER CENT IN 2019-20; 44.4 PER CENT IN 2020-21; 39.4 PER CENT IN 2021-22

NEVER CONFUSE THE PUBLICATION DATE WITH THE ACCOUNTING YEAR A HIGH SHARE SIGNALS WEAK PUBLIC PROVISION, NOT HIGH

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims	SEQUENCE → 47.1 per cent in 2019-20; 44.4 per cent in 2020-21; 39.4 per cent in 2021-22	MEANING → a high share signals weak public provision, not high utilisation
HEALTH ACCOUNTS → official estimates of public, private and out-of-pocket spending	RULE → never confuse the publication date with the accounting year	HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims
SOCIAL UNIT / DEFINITION - HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims - HEALTH ACCOUNTS → official estimates of public, private and out-of-pocket spending	DECISIVE DISTINCTION - SEQUENCE → 47.1 per cent in 2019-20; 44.4 per cent in 2020-21; 39.4 per cent in 2021-22 - RULE → never confuse the publication date with the accounting year	EXAM TRAP / LIMIT - MEANING → a high share signals weak public provision, not high utilisation - HEALTH AUTHORITY → implements the hospitalisation pillar; empanels and pays claims

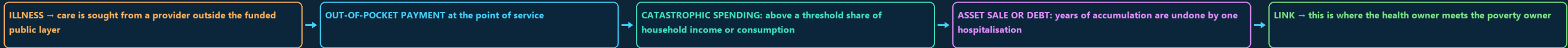
ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → never confuse the publication date with the accounting year.

05

STAGE 05 IMPOVERISHMENT MECHANISM

IMPOVERISHMENT MECHANISM CARE IS SOUGHT FROM A PROVIDER OUTSIDE THE FUNDED OUT-OF-POCKET PAYMENT AT THE POINT OF SERVICE CATASTROPHIC SPENDING ABOVE A THRESHOLD SHARE OF HOUSEHOLD INCOME OR CONSUMPTION ASSET SALE OR DEBT

YEARS OF ACCUMULATION ARE UNDONE BY ONE HOSPITALISATION THIS IS WHERE THE HEALTH OWNER MEETS THE POVERTY



SOCIAL STRUCTURE / CONCEPT	MECHANISM OF CHANGE	CONSEQUENCE / LIMIT
- ILLNESS → care is sought from a provider outside the funded public layer - OUT-OF-POCKET PAYMENT at the point of service	- CATASTROPHIC SPENDING: above a threshold share of household income or consumption - ASSET SALE OR DEBT: years of accumulation are undone by one hospitalisation	- LINK → this is where the health owner meets the poverty owner - ILLNESS → care is sought from a provider outside the funded public layer

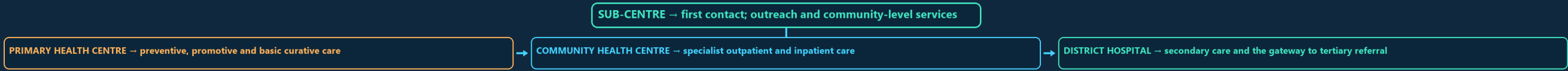
ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: LINK → this is where the health owner meets the poverty owner.

06

STAGE 06 REFERRAL SPINE

REFERRAL SPINE FIRST CONTACT; OUTREACH AND COMMUNITY-LEVEL SERVICES PRIMARY HEALTH CENTRE PREVENTIVE, PROMOTIVE AND BASIC CURATIVE CARE COMMUNITY HEALTH CENTRE SPECIALIST OUTPATIENT AND INPATIENT CARE DISTRICT HOSPITAL

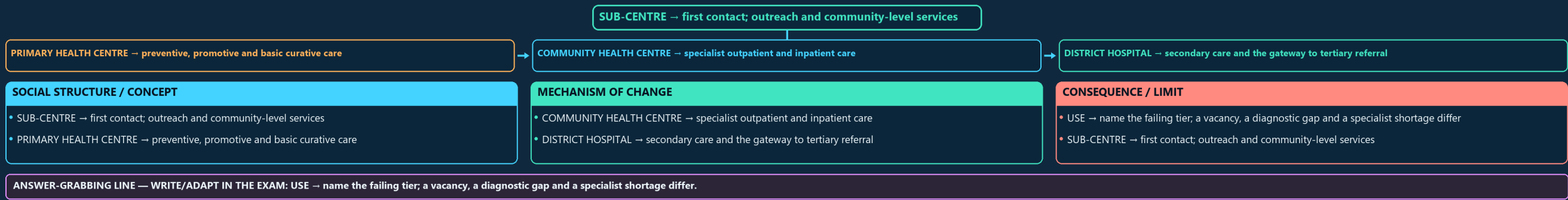
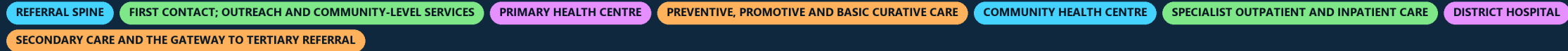
SECONDARY CARE AND THE GATEWAY TO TERTIARY REFERRAL



SOCIAL STRUCTURE / CONCEPT	MECHANISM OF CHANGE	CONSEQUENCE / LIMIT
- SUB-CENTRE → first contact; outreach and community-level services - PRIMARY HEALTH CENTRE → preventive, promotive and basic curative care	- COMMUNITY HEALTH CENTRE → specialist outpatient and inpatient care - DISTRICT HOSPITAL → secondary care and the gateway to tertiary referral	- USE → name the failing tier; a vacancy, a diagnostic gap and a specialist shortage differ - SUB-CENTRE → first contact; outreach and community-level services

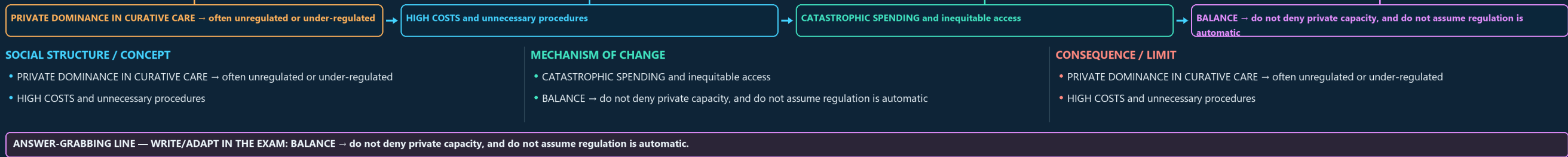
06

STAGE 06 REFERRAL SPINE



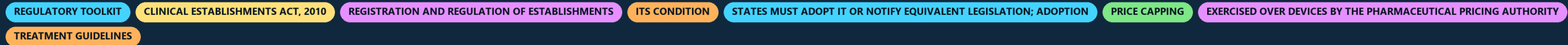
07

STAGE 07 MARKETISATION AND ITS HARMS



08

STAGE 08 REGULATORY TOOLKIT



SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments	PRICE CAPPING → exercised over devices by the pharmaceutical pricing authority	RULE → name the adoption gap a regulatory recommendation is meant to close
ITS CONDITION → States must adopt it or notify equivalent legislation; adoption varies	TREATMENT GUIDELINES → promote rational care and cost control; uptake varies	CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments
SOCIAL UNIT / DEFINITION - CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments - ITS CONDITION → States must adopt it or notify equivalent legislation; adoption varies	DECISIVE DISTINCTION - PRICE CAPPING → exercised over devices by the pharmaceutical pricing authority - TREATMENT GUIDELINES → promote rational care and cost control; uptake varies	EXAM TRAP / LIMIT - RULE → name the adoption gap a regulatory recommendation is meant to close - CLINICAL ESTABLISHMENTS ACT, 2010 → registration and regulation of establishments

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: RULE → name the adoption gap a regulatory recommendation is meant to close.

09

STAGE 09 FINANCING FAMILIES AND CHAIN



SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system	CHAIN → revenue collection to risk pooling to strategic purchasing to provision	TRAP → calling the tax-funded pillar a contributory insurance scheme
TAX-FUNDED → general revenue, as in the health mission and the hospitalisation pillar	GAP → insurance leaves prevention, medicines and outpatient spending unresolved	SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system
SOCIAL UNIT / DEFINITION - SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system - TAX-FUNDED → general revenue, as in the health mission and the hospitalisation pillar	DECISIVE DISTINCTION - CHAIN → revenue collection to risk pooling to strategic purchasing to provision - GAP → insurance leaves prevention, medicines and outpatient spending unresolved	EXAM TRAP / LIMIT - TRAP → calling the tax-funded pillar a contributory insurance scheme - SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: TRAP → calling the tax-funded pillar a contributory insurance scheme.

STAGE 09 FINANCING FAMILIES AND CHAIN

- FINANCING FAMILIES AND CHAIN
- SOCIAL HEALTH INSURANCE
- CONTRIBUTION-LINKED POOLING, AS IN THE EMPLOYEES SYSTEM
- GENERAL REVENUE, AS IN THE HEALTH MISSION AND THE
- REVENUE COLLECTION TO RISK POOLING TO STRATEGIC PURCHASING TO
- INSURANCE LEAVES PREVENTION, MEDICINES AND OUTPATIENT SPENDING UNRESOLVED
- CALLING THE TAX-FUNDED PILLAR A CONTRIBUTORY INSURANCE SCHEME

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
SOCIAL HEALTH INSURANCE → contribution-linked pooling, as in the employees system	CHAIN → revenue collection to risk pooling to strategic purchasing to provision	TRAP → calling the tax-funded pillar a contributory insurance scheme
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ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: TRAP → calling the tax-funded pillar a contributory insurance scheme.

STAGE 10 COMMUNITY HEALTH MAPPING

- COMMUNITY HEALTH MAPPING
- REACH THE UNDERSERVED FIRST RATHER THAN THE EASIEST TO
- VILLAGE HEALTH, SANITATION AND NUTRITION COMMITTEES
- INTER-SECTORAL ACTION
- WATER, SANITATION, NUTRITION AND GENDER EQUALITY
- APPROPRIATE TECHNOLOGY
- FRONTLINE HEALTH ACTIVISTS AND AUXILIARY NURSE MIDWIVES
- THE UPGRADED WELLNESS CENTRE AS THE NAMED INDIAN DELIVERY

SOCIAL UNIT / DEFINITION	DECISIVE DISTINCTION	EXAM TRAP / LIMIT
EQUITY → reach the underserved first rather than the easiest to reach	INTER-SECTORAL ACTION → water, sanitation, nutrition and gender equality	INSTITUTION → the upgraded wellness centre as the named Indian delivery point
PARTICIPATION → village health, sanitation and nutrition committees	APPROPRIATE TECHNOLOGY → frontline health activists and auxiliary nurse midwives	EQUITY → reach the underserved first rather than the easiest to reach
SOCIAL UNIT / DEFINITION - EQUITY → reach the underserved first rather than the easiest to reach - PARTICIPATION → village health, sanitation and nutrition committees	DECISIVE DISTINCTION - INTER-SECTORAL ACTION → water, sanitation, nutrition and gender equality - APPROPRIATE TECHNOLOGY → frontline health activists and auxiliary nurse midwives	EXAM TRAP / LIMIT - INSTITUTION → the upgraded wellness centre as the named Indian delivery point - EQUITY → reach the underserved first rather than the easiest to reach

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: INSTITUTION → the upgraded wellness centre as the named Indian delivery point.

STAGE 11 CAPACITY LIMITS AND ANSWER SPINE

- CAPACITY LIMITS AND ANSWER SPINE
- UNION FINANCING AND STANDARDS DEPEND ON STATE DELIVERY CAPACITY
- WORKFORCE VACANCIES AND UNEVEN CAPACITY LIMIT IDENTICAL NATIONAL SCHEMES
- MENTAL HEALTH
- A STATUTORY RIGHT AND A TELE-SERVICE STILL NEED DISTRICT
- NAME THE COVERAGE DIMENSION TESTED; BUILD
- PROVISION PLUS REGULATION
- GRADED VERDICT WITH A DASHBOARD-COUNT CAVEAT AND A REVERSAL

CONDITION → Union financing and standards depend on State delivery capacity	GAP → workforce vacancies and uneven capacity limit identical national schemes	MENTAL HEALTH → a statutory right and a tele-service still need district professionals	OPEN → name the coverage dimension tested; BUILD → provision plus regulation	CLOSE → graded verdict with a dashboard-count caveat and a reversal condition
SOCIAL STRUCTURE / CONCEPT - CONDITION → Union financing and standards depend on State delivery capacity - GAP → workforce vacancies and uneven capacity limit identical national schemes	MECHANISM OF CHANGE - MENTAL HEALTH → a statutory right and a tele-service still need district professionals - OPEN → name the coverage dimension tested; BUILD → provision plus regulation		CONSEQUENCE / LIMIT - CLOSE → graded verdict with a dashboard-count caveat and a reversal condition - CONDITION → Union financing and standards depend on State delivery capacity	

ANSWER-GRABBING LINE — WRITE/ADAPT IN THE EXAM: CLOSE → graded verdict with a dashboard-count caveat and a reversal condition.

EXTRA SUBORDINATE ENRICHMENT — ONLY AFTER THE COMPLETE CORE

- OPTIONAL ADVANCED ENRICHMENT
- EXPAND HWC/AAM FOR COMPREHENSIVE PRIMARY CARE — REDUCES NEED
- STRENGTHEN NHM (FREE DRUGS, DIAGNOSTICS, WORKFORCE) — IMPROVES PUBLIC
- PM-JAY FOR SECONDARY/TERTIARY COVER — SHIFTS HOSPITALISATION COST FROM
- REGULATORY ENFORCEMENT
- CLINICAL ESTABLISHMENTS ACT, PRICE CAPS (NPPA FOR
- PUBLIC-PRIVATE PARTNERSHIPS WITH ACCOUNTABILITY
- EMPANEL PRIVATE PROVIDERS

OPTIONAL RIGHTS AND CAPABILITY DEPTH - Expand HWC/AAM for comprehensive primary care — reduces need for - Strengthen NHM (free drugs, diagnostics, workforce) — improves public - PM-JAY for secondary/tertiary cover — shifts hospitalisation cost from - Regulatory enforcement: Clinical Establishments Act, price caps (NPPA for	INDICATOR, ROUND AND STATUS LIMIT - Public-private partnerships with accountability: empanel private providers - NHP 2017: Policy, not statute; sets vision for UHC, primary-care-led - NHM (NRHM + NUHM): Centrally-sponsored scheme; provides flexi-pool funds, - Ayushman Bharat - HWC/AAM: Target: 1.5 lakh HWCs (upgraded SCs/PHCs);	COMPARATIVE WELFARE-DESIGN NUANCE - Ayushman Bharat - PM-JAY: Its core route uses notified eligibility criteria - CAUTION: Clinical Establishments Act adoption: Some states have adopted or enacted 2024 Q17: The question directly tests the state's role against - PM-JAY 70+ expansion: The all-citizen 70+ route is operational, regardless of
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OPTIONAL STATUS — This optional depth is unnecessary for a competent core answer; use it only for added nuance after the complete core has been written.